MEDIBANK

Healthcare Intelligence Briefing

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National Health Survey, 2022	ABS	Not stated	26 Sep 2026
National Study of Mental Health and Wellbeing, 2020-2022	ABS	26 Jul 2024	26 Sep 2026
Diabetes: Australian facts	AIHW	13 May 2026	26 Sep 2026
International comparison - Hospitals	AIHW	18 May 2026	26 Sep 2026

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Insights

HIGH PRIORITY *ABS · Clinical*

Young women aged 16-24 have the highest prevalence of 12-month mental disorders of any age-sex group, at 45.5%, driven largely by anxiety disorders (40.4%), and they also have the highest rates of self-harm in the last 12 months (8.7%).

Evidence "Almost half of females (45.5%) aged 16–24 years and a third of males (32.4%) aged 16–24 years had a 12-month mental disorder."
Section: Prevalence of 12-month mental disorders - Prevalence of 12-month mental disorders by age and sex · Source: National Study of Mental Health and Wellbeing, 2020-2022

Why this priority Relevant through high mental-disorder burden and self-harm in young women; 45.5% prevalence and 8.7% self-harm are material current clinical/safety impacts. Clinical significance and actionability drive High, although utilisation, cost and trend data are not supplied.

HIGH PRIORITY *ABS · Clinical*

Of the 644,600 Australians who had suicidal thoughts or behaviours in the last 12 months, 74.9% had a 12-month mental disorder, indicating that a quarter of people with recent suicidality do not meet criteria for the surveyed mental disorders.

Evidence "Of the 644,600 people who reported any suicidal thoughts or behaviours in the last 12 months, three in four (74.9%) had a 12-month mental disorder."
Section: Lived experience of suicide · Source: National Study of Mental Health and Wellbeing, 2020-2022

Why this priority Relevant because recent suicidal thoughts or behaviours affect a large population and represent a severe clinical/safety signal. Clinical significance, affected population and actionability drive High, although service use, cost and trend evidence are not supplied.

HIGH PRIORITY *AIHW · Pharmaceutical*

GLP-1 analogue prescriptions dispensed through the PBS nearly doubled in three years, rising 87% from 1.7 million in 2021-22 to 3.2 million in 2024-25, making it the fastest-growing class of diabetes medicines and making semaglutide (Ozempic) the seventh most costly PBS-subsidised medicine in 2024-25.

Evidence "The largest increase among the dispensed diabetes medicines was for glucagon-like peptide-1 (GLP-1) analogues which rose by 87% from 1.7 million in 2021–22 to 3.2 million in 2024–25."
Section: Summary - Key facts · Source: Diabetes: Australian facts

Why this priority Relevant because it shows a rapid, major increase in PBS-dispensed GLP-1 analogue use, with semaglutide now the seventh most costly PBS-subsidised medicine, a material pharmaceutical utilisation and cost development. High Priority is driven by High utilisation_demand, Large magnitude_of_change, High cost_implications and Strong trend_continuity; private-health impact remains indirect.

HIGH PRIORITY *AIHW · Clinical*

Almost 1 in 5 women (18%) who gave birth in hospital in 2023-24 were diagnosed with gestational diabetes, a rate that has doubled since 2012-13 (from 9.3% to 18.9%), with the highest incidence (32%) among women aged 45-49.

Evidence "Almost 1 in 5 (18% or 49,800) females aged 15–49 who gave birth in hospital in 2023–24 had been diagnosed with gestational diabetes during their pregnancy, according to the National Hospital Morbidity Database (NHMD)."
Section: Summary - Gestational diabetes · Source: Diabetes: Australian facts

Caveat The rising incidence is likely driven by several factors including increasing maternal age, higher rates of maternal overweight and obesity, and a growing proportion of higher risk ethnic groups in the population. The introduction of new diagnostic guidelines across all states and territories between 2011 and 2013 is also likely to have contributed towards the increasing rates.

Why this priority Relevant because gestational diabetes now affects almost one in five hospital births and the prevalence has doubled over about a decade, a substantial current obstetric and population-health development. Large population_affected, Large magnitude_of_change and Strong trend_continuity drive High Priority; the caveat notes diagnostic guideline changes may partly explain the rise.

MEDIUM PRIORITY *ABS · Clinical*

Three-quarters (74.5%) of adults with high measured blood pressure did not report having hypertension, indicating a large pool of undiagnosed or unreported hypertension in the community.

Evidence "Three in four (74.5%) adults with high measured blood pressure did not report having hypertension."
Section: Hypertension and high measured blood pressure · Source: National Health Survey, 2022

Why this priority This is a relevant population-health care-gap signal: a large share of adults with high measured blood pressure did not report hypertension. Priority is driven mainly by the notable measured-versus-reported comparator and moderate clinical significance, while utilisation, costs and population scale are not quantified; the main caveat is that not reporting hypertension does not definitively prove undiagnosed disease, and no Medibank-specific prevalence is supplied.

MEDIUM PRIORITY *ABS · Clinical*

Diabetes prevalence has slowly increased from 3.3% in 2001 to 5.3% in 2022, with 1.3 million people now living with the condition.

Evidence
"Diabetes prevalence has slowly increased from 3.3% in 2001 to 5.3% in 2022."
Section: Diabetes · Source: National Health Survey, 2022

Why this priority
This is relevant through a substantial and sustained chronic-disease burden: diabetes prevalence increased from 3.3% in 2001 to 5.3% in 2022, affecting an explicitly large population of 1.3 million. The strong trend, large affected population, and moderate magnitude of change drive Medium Priority, but the evidence is national and does not establish Medibank-specific prevalence, utilisation, or cost impact.

MEDIUM PRIORITY *ABS · Clinical*

Only 45.1% of Australians with a 12-month mental disorder saw a health professional for their mental health in 2020-2022, leaving a majority (54.9%) without any professional consultation, with the gap most pronounced among males (36.4% consulted) and older adults aged 65-85 (35.1% consulted).

Evidence
"There were 4.3 million Australians aged 16–85 years with a 12-month mental disorder. Of these, 1.9 million (45.1%) saw a health professional for their mental health in 2020–2022:"
Section: Use of services - Consultations with health professionals for mental health · Source: National Study of Mental Health and Wellbeing, 2020-2022

Why this priority
Supplied evidence connects to large mental-health burden and service use, with a majority of affected Australians not consulting a health professional. Clinical significance, utilisation/demand and affected population drive the assessment, but the insight is a cross-sectional snapshot without cost or trend data, so Medium Priority is appropriate.

MEDIUM PRIORITY *ABS · Pharmaceutical*

Among Australians with a 12-month mental disorder, only 33.3% were dispensed a PBS-subsidised mental health-related medication, and this proportion rises sharply with age: 25.1% for those aged 16-34, 39.3% for 35-64, and 51.0% for those aged 65-85.

Evidence
"There were 4.3 million Australians aged 16–85 years with a 12-month mental disorder. Of these, 1.4 million (33.3%) were dispensed at least one PBS subsidised mental health-related medication in the 12 months prior to interview:"
Section: Mental health-related medications - Mental health-related medications dispensed to people with a 12-month mental disorder · Source: National Study of Mental Health and Wellbeing, 2020-2022

Caveat
PBS data cannot determine what condition a medication was prescribed for and only shows that a medication has been dispensed.

Why this priority
Relevant through mental-health-related medication use in a large affected population; the age gradient from 25.1% to 51.0% is a notable comparator. Utilisation/demand and affected population drive Medium, but the PBS caveat prevents attribution to diagnosed conditions and no cost or trend data are supplied.

MEDIUM PRIORITY *ABS · Clinical*

People who identify as Gay, Lesbian, Bisexual, or use a different term have a 12-month mental disorder prevalence of 58.7%, nearly three times the rate of heterosexual people (19.9%), with anxiety disorders affecting half (50.3%) of this group.

Evidence "People who described their sexual orientation as Gay or Lesbian, Bisexual or who used a different term had higher rates of 12-month mental disorders (58.7%) than people who described their sexual orientation as heterosexual (19.9%)"
Section: Prevalence of 12-month mental disorders - Prevalence of 12-month mental disorders by different populations · Source: National Study of Mental Health and Wellbeing, 2020-2022

Why this priority Relevant due very high mental-disorder prevalence in LGB+ respondents and a nearly three-fold disparity relative to heterosexual respondents. Clinical significance and magnitude of difference support Medium, but no utilisation, cost, trend or direct private-health evidence is supplied.

MEDIUM PRIORITY *ABS · Pharmaceutical*

A substantial proportion of people dispensed mental health-related medications have no lifetime mental disorder: 34.9% of all those dispensed any such medication, and 56.1% of those dispensed hypnotics and sedatives.

Evidence "Of the 3.5 million people aged 16–85 years who were dispensed at least one PBS subsidised mental health-related medication in the 12 months prior to interview, 40.7% had a 12-month mental disorder, 24.2% had a lifetime disorder with no 12-month symptoms, and the remainder (34.9%) had no lifetime disorder."
Section: Mental health-related medications - People who were dispensed a mental health-related medication · Source: National Study of Mental Health and Wellbeing, 2020-2022

Caveat PBS data cannot determine what condition a medication was prescribed for and only shows that a medication has been dispensed.

Why this priority Relevant through widespread mental-health-related medication use and the notable proportion of dispensing among people without a lifetime disorder, particularly hypnotics and sedatives. Utilisation and affected population drive Medium, but the PBS caveat and absence of cost or trend data reduce certainty.

MEDIUM PRIORITY *AIHW · Clinical*

Diabetes contributes to 11% of all deaths in Australia (around 20,800 deaths in 2024), with the death rate 2.4 times higher among people in the lowest socioeconomic areas compared to the highest, and 4.0 times higher for First Nations people than non-Indigenous Australians.

Evidence "Diabetes remains one of the 10 leading causes of death in Australia and was the underlying cause of around 6,000 deaths in 2024. However, it contributed to around 20,800 deaths (11% of all deaths) when associated causes are also considered (77 deaths per 100,000 population)."
Section: Summary - Deaths · Source: Diabetes: Australian facts

Caveat The increase between 2021 and 2022 should be interpreted in the context of higher overall mortality in 2022, with two-thirds of excess deaths being associated with COVID-19 (ABS 2023a).

Why this priority Relevant because diabetes contributes to 11% of all deaths and there are large mortality disparities by socioeconomic status and Indigenous status, a severe population-health burden. Clinical_significance and the notable comparator disparities drive Medium Priority, but no trend, utilisation, or direct private-health impact is supplied.

MEDIUM PRIORITY *AIHW · Clinical*

In 2023-24, there were 7,000 lower limb amputations and 3,300 weight loss procedures among people with type 2 diabetes, with amputation rates 3.9 times higher in males and obesity surgery rates twice as high in females.

Evidence "In 2023–24, there were 3,300 weight loss procedures and 7,000 lower limb amputations undertaken for people with type 2 diabetes."
Section: Summary - Hospitalisations and procedures · Source: Diabetes: Australian facts

Why this priority Relevant because the evidence quantifies serious diabetes-related hospital procedures, including 7,000 lower limb amputations and 3,300 weight loss procedures in one year, with large sex-based differences. Clinical_significance, utilisation_demand and the comparator signal drive Medium Priority; no trend or cost evidence is supplied and private-hospital status is not specified.

MEDIUM PRIORITY *AIHW · Clinical*

First Nations adults are 3.2 times as likely to be living with diabetes as non-Indigenous Australians, with 16% of First Nations adults affected, and they have 3.5 times the hospitalisation rate and 4.0 times the death rate for diabetes.

Evidence "Around 92,000 First Nations adults (16%) were living with diabetes in 2022–24 (ABS 2025b). First Nations adults were 3.2 times as likely as non–Indigenous adults to have diabetes, after adjusting for age (ABS 2025a; ABS 2025b)."
Section: Summary - Variation by priority population groups · Source: Diabetes: Australian facts

Why this priority Relevant because First Nations adults have substantially higher diabetes prevalence, hospitalisation and mortality, representing a major health-equity burden. Clinical_significance and the large comparator disparities drive Medium Priority; no trend and no direct private-health connection are supplied.

MEDIUM PRIORITY *AIHW · Clinical*

Only 30% of people with diabetes attending services for diabetes care in 2024 met the target HbA1c of ≤7%, with median HbA1c at 7.9% (7.7% for type 1 and 8.0% for type 2), indicating suboptimal glycaemic control in the majority of patients.

Evidence "According to data from the Australian Diabetes Clinical Quality Registry, in 2024, glycaemic control targets were inadequately met, with the median HbA1c for both type 1 diabetes (7.7%) and type 2 diabetes (8.0%) being above target. Similar proportions of people with type 1 diabetes (31%) and type 2 diabetes (29%) met the target HbA1c of ≤7% (or ≤53 mmol/mol)."
Section: Pathology - Page highlights · Source: Diabetes: Australian facts

Caveat Data are from the Australian Diabetes Clinical Quality Registry, which may not be representative of all people with diabetes in Australia.

Why this priority Relevant because most patients attending diabetes services are not meeting glycaemic targets, indicating suboptimal diabetes care quality. The target benchmark and clinical significance drive Medium Priority, but the registry evidence is caveated as potentially non-representative and no utilisation, trend, or cost impact is supplied.

MEDIUM PRIORITY *AIHW · Clinical*

In 2023-24, women with pre-existing diabetes had a caesarean section rate of 70.8% and a pre-term birth rate of 26.3%, compared to 42.0% and 7.4% respectively for women without diabetes, indicating substantially higher obstetric intervention and risk.

Evidence "In 2023–24, compared to mothers without diabetes, mothers living with pre-existing diabetes (and to a lesser extent, gestational diabetes), were more prone to complications during pregnancy and intervention in childbirth. After adjusting for differences in the age structure of the populations, such complications and interventions include caesarean section, induced labour, preterm birth, maternal care for excessive foetal growth and pre-eclampsia (Figure 1)."
Section: Diabetes-related complications - Short-term outcomes · Source: Diabetes: Australian facts

Why this priority Relevant because women with pre-existing diabetes have markedly higher caesarean and preterm birth rates, indicating substantial obstetric risk and intervention. Clinical_significance and the large comparator differences drive Medium Priority; no trend, cost, or private-hospital breakdown is supplied.

MEDIUM PRIORITY *AIHW · Clinical*

Australia's caesarean section rate in 2024-25 was 42.7 per 100 live births, well above the OECD average of 30.5, a gap of over 12 percentage points.

Evidence "In 2024–25, Australia’s rate of caesarean sections per 100 live births (42.7) was higher than the OECD average (30.5)."
Section: Number of caesarean sections per 100 live births · Source: International comparison - Hospitals

Caveat The data reported by the OECD may be based on different reporting periods.

Why this priority Relevant because caesarean section is a major hospital maternity procedure, and Australia's rate is markedly above the OECD average (42.7 vs 30.5 per 100 live births), indicating substantial surgical utilisation and a broad affected population. The absence of trend or cost data and the OECD reporting-period caveat limit this to a meaningful current benchmark signal rather than a higher-priority development.

MEDIUM PRIORITY *AIHW · Clinical*

Australia's knee replacement rate in 2024-25 was 216.9 per 100,000 population, 48% above the OECD average of 146.1, while hip replacement was below the OECD average (176.9 vs 196.1).

Evidence "Australia’s rate of knee replacement surgery was above the OECD average (216.9 and 146.1 per 100,000 population, respectively)."
Section: Number of hip and knee replacement surgeries per 100,000 population · Source: International comparison - Hospitals

Caveat The data reported by the OECD may be based on different reporting periods.

Why this priority Relevant because knee replacement is a major surgical procedure, and Australia's rate is 48% above the OECD average, providing a substantial current benchmark and utilisation signal with a large affected population. No demonstrated trend, cost evidence, or direct private-insurance impact is supplied, and the OECD reporting-period caveat qualifies the comparison, so the finding is a Medium-priority comparator issue.

WATCHLIST *ABS · Watchlist*

In 2020-2022, 4.8% of Australians (945,000) used digital technologies to access mental health services other than consultations, with usage highest among younger people (8.2% of 16-34 year olds) and females (6.0%).

Evidence "Nearly 945,000 Australians (4.8%) aged 16–85 years used these methods to access other services for their mental health in the 12 months prior to interview:"
Section: Other services for mental health accessed using digital technologies · Source: National Study of Mental Health and Wellbeing, 2020-2022

Why this priority Relevant through digital mental-health service access, with higher use among younger people and females. It is a plausible emerging service channel, but no trend, cost, direct private-health or clear emergence evidence is supplied, so it is best placed on the Watchlist.

INFORMATIONAL *ABS · Pharmaceutical*

83.0% of people with one or more chronic conditions were dispensed at least one PBS medication in the six months before or after their interview, compared to 68.6% of the total population.

Evidence "Over eight in ten (83.0%) of people with one or more chronic conditions were dispensed at least one PBS medication."
Section: Pharmaceutical Benefits Scheme (PBS) medications · Source: National Health Survey, 2022

Why this priority This is a relevant medication-utilisation snapshot showing high PBS dispensing among people with chronic conditions compared with the total population. Utilisation/demand is high and evidence is strong, but there is no demonstrated trend, material change, cost data, or direct private-health impact, so it remains informative context rather than a priority development.

INFORMATIONAL *ABS · Clinical*

People in Tasmania were more likely to have at least one chronic condition (58.4%) than any other state or territory.

Evidence "People in Tasmania were more likely to have at least one chronic condition (58.4%) than any other state or territory"
Section: State and territory findings · Source: National Health Survey, 2022

Why this priority This is relevant as a regional chronic-disease burden signal, with Tasmania having the highest state/territory prevalence of at least one chronic condition at 58.4%. However, it is a single cross-sectional comparison without a demonstrated trend, quantified difference, utilisation, cost, or direct private-health impact, so it provides useful contextual information rather than a material current development.

INFORMATIONAL *ABS · Clinical*

In 2020-2022, 8.4% of Australians (1.7 million) had both a 12-month mental disorder and a selected physical condition, and this comorbidity is more common in females (10.5%) than males (6.3%).

Evidence "1.7 million people (8.4%) had both a 12-month mental disorder and a physical condition"
Section: Comorbidity of mental disorders and physical conditions · Source: National Study of Mental Health and Wellbeing, 2020-2022

Caveat Comorbidity data presented in this figure does not incorporate hierarchy rules which give precedence to diagnosis of one mental disorder over another.

Why this priority Relevant as a population-level mental–physical comorbidity snapshot, but it is static cross-sectional data without service use, cost or trend evidence. The sex difference is contextual, so no material current development is established and the priority is Informational.

INFORMATIONAL *ABS · Clinical*

The prevalence of 12-month mental disorders is highest in the Northern Territory (28.8%) and lowest in New South Wales (19.5%), but the wide confidence intervals for smaller states mean these differences are not statistically significant.

Evidence "| NT | 28.8 | 15.2 | 42.4 |"
Section: Interpreting sampling error: 12-month mental disorders by state/territory · Source: National Study of Mental Health and Wellbeing, 2020-2022

Caveat The NSMHW sample was designed to provide reliable national-level estimates. While estimates are available for each state and territory, the sample size does not allow for detailed analysis and care should be taken if comparing states and territories.

Why this priority Relevant through national mental-health prevalence, but the state/territory differences are not statistically significant and the sample is not designed for detailed state comparison. Low evidence strength and actionability, along with no trend or cost data, make this Informational.

INFORMATIONAL *ABS · Clinical*

In 2020-2022, 17.4% of Australians (3.4 million) saw a health professional for their mental health, but only 12.4% saw a GP and 7.8% saw a psychologist, indicating that GPs are the most common point of contact for mental health care.

Evidence "12.4% of people saw a general practitioner for their mental health and 7.8% saw a psychologist."
Section: Use of services - Consultations with health professionals for mental health · Source: National Study of Mental Health and Wellbeing, 2020-2022

Why this priority Relevant due substantial mental-health service utilisation and GP/psychologist care patterns. However, this is a static cross-sectional utilisation snapshot without trend, cost or private-specific evidence; absolute utilisation alone does not establish a material current development, so the priority is Informational.

INFORMATIONAL *AIHW · Clinical*

In 2024-25, only 25% of tonsillectomies in Australia were performed as same-day procedures, compared with the OECD average of 39%, while 98% of cataract surgeries were same-day, well above the OECD average of 76%.

Evidence "the proportion of tonsillectomies that were performed on a same-day basis in Australia (25%) was lower than the OECD average (39%)."
Section: Proportion of surgeries performed on a same-day basis · Source: International comparison - Hospitals

Caveat The data reported by the OECD may be based on different reporting periods.

Why this priority Relevant because same-day surgery patterns affect hospital care and potential resource use, with Australia's tonsillectomy same-day rate below the OECD average (25% vs 39%). The finding is a moderate benchmark signal, but without trend, clinical outcome, or cost evidence and with a smaller affected population, it does not establish a sufficiently material current development.

INFORMATIONAL *AIHW · Clinical*

In 2024-25, Australia performed 95% of appendicectomies laparoscopically, compared with the OECD average of 79%, and 53% of inguinal herniorrhaphies laparoscopically, compared with 35%.

Evidence "95% of appendicectomies in Australia were performed laparoscopically, compared with the OECD average (79%)"
Section: Proportion of selected surgical procedures that were performed laparoscopically · Source: International comparison - Hospitals

Caveat The data reported by the OECD may be based on different reporting periods.

Why this priority Relevant because laparoscopic surgical rates concern hospital procedure pathways and private hospital care, and Australia is above OECD averages for both reported procedures. There is no evidence of an adverse clinical, cost, or utilisation development or direct private-insurance impact, and the international comparability caveat limits the finding to useful contextual benchmark information.